

Clinical Study Summary Report (CSSR)

Document Status: Draft | Version: 1.0 | Date: 02-Apr-2026

1. Administrative & Study Identification

Full Study Title: A Multicentre, Interventional, Phase IV, Open-label, Study to Evaluate the Safety of Palivizumab in Children Less Than 24 Months of Age With High Risk of Severe Respiratory Syncytial Virus (RSV) Disease **Short Title/Acronym:** Study of Palivizumab in Children With High Risk of Severe Respiratory Syncytial Virus (RSV) Disease **Protocol Number:** [Internal Study ID / Not Published] **NCT Number:** NCT06851806 **Posting ID:** 201787187103731964 **Sponsor Name / Company:** AstraZeneca **Investigational Product:** Palivizumab (Trade Name: Synagis | ATC Code: J06BD01) **EMA URL:** https://www.ema.europa.eu/en/documents/product-information/synagis-epar-product-information_en.pdf **Phase of Development:** NA **Trial Status:** NOT_YET_RECRUITING **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective **Primary Objective:** To assess the safety of palivizumab prophylaxis when administered to preterm infants (≤ 35 wGA), children with bronchopulmonary dysplasia (BPD) (< 24 months), and/or children with haemodynamically significant congenital heart disease (CHD) (< 24 months). **Secondary Objectives:** * To determine the percentage of participants with Adverse Events (AEs) and Serious Adverse Events (SAEs). * To assess the nature, incidence, and severity of all AEs including unexpected adverse drug reactions. * To determine the percentage of participants with AEs that lead to study intervention discontinuation. * To evaluate biomarkers that correlate with RSV disease severity in high-risk children.

2.2 Background & Rationale To address the clinical need for safe and effective prophylaxis against Respiratory Syncytial Virus (RSV) in high-risk demographic populations. Specifically, the study generates data on the safety profile of palivizumab intramuscular (IM) injection for the prevention of severe Lower Respiratory Tract Disease (LRTD) in Indian infants and children who are vulnerable due to prematurity or cardiopulmonary conditions.

3. Study Design & Methodology

3.1 Design Framework **Study Type:** Interventional **Control Type:** Single-arm (Single Group Assignment) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target **NS:** 138 participants
Number of Sites: Multicentre (India) **Estimated Study Start:** 30-Nov-2025 **Estimated Primary Completion:** 28-Feb-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Participants at risk of severe RSV disease defined as fulfilling at least one of the following: 1. Infants born ≤ 35 wGA and are < 6 months of age at enrolment. 2. Children < 24 months of age at enrolment and requiring treatment for BPD within the last 6 months. 3. Children < 24 months of age and with haemodynamically significant CHD. 2. Written informed consent obtained from the participant's parent(s)/legal guardian and the participant's parent(s)/legal guardian is able to understand and comply with the requirements of the protocol including follow-up visits as judged by the investigator.

4.2 Exclusion Criteria Participants are excluded from the study if any of the following criteria apply: 1. Hospitalisation at the time of enrolment, unless the discharge is expected within 30 days of the time of enrolment. 2. Required mechanical ventilation (including continuous positive airway pressure) or other mechanical respiratory or cardiac support at the time of enrolment. 3. Anticipated cardiac surgery within 2 weeks after enrolment. 4. Anticipated survival of < 6 months after enrolment in the trial. 5. Active LRTD, including RSV infection at the time of enrolment and/or study intervention administration. 6. Any fever ($\geq 38.0^{\circ}\text{C}$) or acute illness within 7 days prior to investigational product administration. 7. Known history of evolving or unstable neurologic disorder. 8. Known history of unstable cardiac or respiratory status, including cardiac defects so severe that survival is not expected or for which cardiac transplantation is planned or anticipated. 9. Known allergy, including to immunoglobulin products, or history of allergic reaction. 10. Receipt of palivizumab or other RSV monoclonal antibodies or any RSV vaccine, including maternal RSV vaccination. 11. Receipt of any monoclonal or polyclonal antibody (for example, hepatitis B immune globulin, intravenous immunoglobulin) or anticipated use during the study. 12. Concurrent enrolment in another interventional study. 13. Children of employees of the sponsor, clinical study site, or any other individuals involved with the conduct of the study, or immediate family members of such individuals. 14. Judgment by the investigator that the participant should not participate in the study if the participant or the participant's parent/legal guardian is unlikely to comply with study procedures, restrictions, and requirements.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Palivizumab 15 mg/kg once a month. Children who undergo cardiac surgery with cardiopulmonary bypass during the study should receive an additional dose of study intervention immediately after surgery when medically stable. **Route of Administration:** Intramuscular (IM) injection. **Duration of Treatment:** Up to 5 injections (Dosed on Days 1, 31, 61, 91, and 121).

5.2 Concomitant & Prohibited Therapy Permitted: Routine medical follow-up and standard care for underlying cardiopulmonary disorders. **Prohibited:** Concomitant administration of other RSV monoclonal antibodies or RSV vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -14 to 0	Informed Consent from parent(s)/legal guardian, Medical History, Baseline Assessments
Baseline	Day 1	First Dose of Palivizumab (15 mg/kg IM), Vitals
Interim	Days 31, 61, 91	Subsequent Doses of Palivizumab, AE/SAE Assessment, Concomitant Medications Review
End of Study	Day 121+	Final Dose (Day 121), Final Follow-up, AE/SAE Reporting, Biomarker Analysis

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Percentage of participants experiencing AEs and SAEs resulting from palivizumab prophylaxis. **Measurement Method:** Clinical assessment and investigator reporting over the study duration.

7.2 Secondary & Exploratory Endpoints * Incidence, nature, and severity of unexpected adverse drug reactions. * Percentage of participants with AEs leading to treatment discontinuation. * Evaluation of specific biomarkers correlating with RSV disease severity.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via HCP interview with parent/guardian and clinical examination at each dosing visit. **Serious Adverse Events (SAEs):** Reported within 24 hours of site awareness. **Data Monitoring Committee (DMC):** Internal safety review managed by the sponsor (AstraZeneca).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all enrolled infants/children who receive at least one dose of the investigational product).

Sample Size Justification: Target sample size of $N=138$ is sufficiently powered to evaluate the safety and tolerability profile in the specific localized demographic (Indian infants/children).

Handling of Missing Data: Last Observation Carried Forward (LOCF) where applicable, or evaluated per protocol.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring to ensure protocol compliance and safety adherence. **Audit Trail:** Data entered via eCRF; standard safety clarification and source data verification forms will be enforced.

11. Study Identifiers & Governance

Primary ID: NCT06851806 **Registry Number:** NCT06851806 **Posting ID:** 201787187103731964 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** Study of Palivizumab in Children With High Risk of Severe Respiratory Syncytial Virus (RSV) Disease **Official Regulatory Title:** A Multicentre, Interventional, Phase IV, Open-label, Study to Evaluate the Safety of Palivizumab in Children Less Than 24 Months of Age With High Risk of Severe Respiratory Syncytial Virus (RSV) Disease

12. Clinical Framework (RSV Prevention Specific)

Primary Condition / Disease Area: Respiratory Syncytial Virus (RSV) **Preferred UMLS Name:** Respiratory Syncytial Virus Infections

Condition Preferred Name: Respiratory syncytial virus (RSV) infection in conditions classified elsewhere and of unspecified site **Semantic Type:** Disease or Syndrome **Definition:** Respiratory Syncytial Virus (RSV) is a common respiratory virus that causes

infections of the lungs and respiratory tract. It is a leading cause of bronchiolitis and pneumonia, especially in infants and young children, but can also affect older adults and individuals with weakened immune systems. Symptoms range from mild, cold-like signs to severe respiratory distress requiring hospitalization.

MeSH Code: D012130 **SNOMED ID:** 36966002 **SNOMED Hierarchy:**

241770007|Viral infectious disease|Viral infectious disease

(disorder) **Diagnosis Threshold:** Preterm infants ≤ 35 wGA, or children < 24 months with BPD/CHD **Medical Methodology:** Passive immunization for prophylaxis **Optimization Target:** Prevention of

severe RSV infection and mitigation of RSV-related hospitalizations

13. Study Procedures & Methodology

Management Pathway: Monthly IM injection administration during RSV vulnerability period. **Education Protocol (HCP):** Site visits and training regarding pediatric IM injection and immediate AE monitoring. **Education Protocol (Patient/Guardian):** Onsite disease education and standard counseling regarding warning signs of AEs/illness. **Quality Audit Mechanism:** Adherence monitoring of the 5-dose monthly schedule and vital signs tracking.

14. Observation & Follow-Up Schedule

Total Duration: Approximately 5 months per subject (evaluating up to 5 injections) **Onsite Visit Cadence:** Day 1, Day 31, Day 61, Day 91, Day 121 **Remote Monitoring Frequency:** Every 4 weeks (Q4W) between clinic visits for safety surveillance. **Checklist Review Interval:** During each monthly administration visit.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				